

Your Body, Their Data:

Reproductive Surveillance and the Long Project of American Control

Alyssa Langmeyer

Abstract

The Supreme Court's decision in *Dobbs v. Jackson Women's Health Organization* activated a pre-existing surveillance infrastructure of reproductive control that has long targeted marginalized bodies. This article argues that the weaponization of reproductive health data is not

a policy gap or technological anomaly, but the modern manifestation of a centuries-old project to regulate bodily autonomy, particularly for women, people of color, and LGBTQ+ individuals. Through analysis of legal statutes, case law, and reported criminal cases, I demonstrate how the convergence of state trigger bans, an unregulated data broker industry, and advanced algorithmic inference operationalizes this project through digital means. HIPAA and similar frameworks fail because they were designed for a world where privacy violations were the exception, not the rule. Today, surveillance is not a bug of the digital economy but its operating system. The article proposes the Digital Bodyguard Act not as a comprehensive solution, but as a necessary intervention that reveals the deeper truth: so long as our legal and political systems remain built on foundations of patriarchy and white supremacy, the state will always find new tools, from archaic laws to digital dragnets, to police, punish, and perpetuate its power.

Introduction: The Post-Dobbs Surveillance Landscape

The regulation of bodies has always been central to the American project. From the commodification of enslaved Black women's wombs (Owens & Fett, 2019) to the forced sterilization of Indigenous (Theobald, 2019) and disabled people (Métraux, 2025), the state has consistently asserted its power to determine whose bodies are autonomous and whose are subject to control. Reproductive surveillance is not a new phenomenon, it is the thread connecting chattel slavery's reproduction of property (Owens & Fett, 2019) to the eugenics movement's reproduction of whiteness (Bailey & Zita, 2007) to today's digital tracking of pregnant people (Bhuiyan, 2025). What has changed is the mechanism. Where control once required physical presence (the plantation overseer, the sterilization surgeon, the clinic door), it now operates through location histories, search queries, and algorithmic inferences that render the body visible to the state at all times. To understand how we arrived at this moment, we must first trace the long history of reproductive regulation in America, then examine the legal foundation that briefly protected reproductive rights, and finally consider the surveillance infrastructure that *Dobbs* activated.

The Historical Roots

Consider what it meant to be a pregnant woman in the antebellum South. Her body was not her own. Under the doctrine of *partus sequitur ventrem*, codified into Virginia law in 1662 and replicated across the colonies, the child followed the condition of the mother, meaning that enslaved women's wombs produced property for their enslavers. Every pregnancy was an economic event, every birth added to the master's wealth. The state enforced this through laws that treated enslaved women as breeding stock and through the violence of the plantation, where overseers asserted control not just over labor but over reproduction itself (Owens & Fett, 2019). This was the most extreme form of a deeper truth: under patriarchy, women's bodies have never fully been their own. The law of coverture, imported from English common law, held that a married woman's identity was "covered" by her husband. She could not own property, sign

contracts, or control her own wages. Her body, her labor, and her children belonged to him (Walsh, 2023). White women were not property in the same way enslaved women were, but they were also not fully sovereign over their own bodies.

Jump to the early twentieth century. The eugenics movement, embraced by leading scientists and legitimized by the Supreme Court in *Buck v. Bell*, argued that the state had not just a right but a duty to prevent the "unfit" from reproducing. Carrie Buck, a young woman incarcerated at the Virginia Colony for Epileptics and Feeble-Minded, was sterilized against her will in 1927. Justice Oliver Wendell Holmes delivered the logic with brutal clarity: "Three generations of imbeciles are enough." (*Buck v. Bell*, 274 U.S. 200 (1927), n.d.) That decision has never been overturned. It authorized the sterilization of some seventy thousand Americans, disproportionately poor, disabled, Indigenous, and Black (Burrus, 2016). But eugenic thinking was never only about race and disability; it was also about gender. Women deemed "unfit" were disproportionately targeted for sterilization because their capacity to bear children was framed as a threat to the nation's genetic future (D. E. Roberts, 2022). Prominent eugenicists argued that educated white women were not reproducing enough while poor and immigrant women reproduced too much (n.d.). This concern reveals how anxieties about women's reproductive choices have always been central to the project of population control. The rationale was simple: some bodies threaten the genetic health of the nation. The state must intervene.

Jump again, to the 1960s and 70s. The welfare rights movement fought against laws that sent caseworkers into the homes of poor mothers, mostly Black, to inspect for the presence of a man. If a man was found, benefits could be cut. The state's logic was that poor women's reproduction required supervision, that their children were suspect, that their sexuality was a matter of public concern (Garrity, 2024). These "man in the house" rules were eventually struck down (*King v. Smith*, 392 U.S. 309 (1968), n.d.), but the impulse behind them never disappeared. For middle-class white women, control took a different form: the denial of contraception and abortion, the expectation that they would leave paid work upon marriage, the medical establishment's routine performance of unnecessary hysterectomies without informed consent (n.d.). Patriarchy does not operate uniformly, but universally. This history of control was briefly interrupted by a legal framework that recognized a constitutional right to privacy and, with it, a right to abortion.

The Legal Foundation

The legal battle for reproductive rights in the United States has historically been fought on the terrain of clinical access and physical liberty, not digital privacy. The 1965 case of *Griswold v. Connecticut* established a "right to privacy" within marital relationships, specifically regarding the use of contraception (*Griswold v. Connecticut*, 1965). This constitutional principle became the bedrock of later reproductive rights cases like *Roe v. Wade* (*Roe v. Wade*, 1973) and *Planned Parenthood v. Casey*, establishing a federal right to abortion (*Planned Parenthood v. Casey*, 1992). Within that legal paradigm, the primary threats were to clinical access, rendering an individual's digital footprint a largely irrelevant concern (Planned Parenthood, n.d.).

This legal foundation began to be dismantled in June 2022 when the Supreme Court overturned *Roe* and *Casey* with *Dobbs v. Jackson Women's Health Organization*, eliminating the federal constitutional right to abortion and returning its regulation to the states (*Dobbs v. Jackson Women's Health Organization*, 2022). This immediately activated a patchwork of laws across the country. In states like Texas and Georgia, so-called "trigger bans," pre-written, dormant statutes designed to take effect upon *Roe's* fall, immediately criminalized abortion with limited exceptions, fundamentally altering the legal landscape for patients and providers (Guarnieri & Nash, 2022). This created a nation where a healthcare decision is a protected right in one state and a prosecutable crime in another. Critically, the *Dobbs* decision opened the door for states to investigate and prosecute individuals, not just providers, for obtaining or seeking an abortion.

The *Dobbs* majority rested its decision on an originalist reading of the Constitution. Justice Alito argued that a right to abortion could not be "deeply rooted in the Nation's history and traditions." Legal scholars have noted the obvious problem with this reasoning: women were excluded from the constitutional drafting process. The "traditions" the Court invoked were those of propertied white men. Justice Thomas's concurrence made the stakes explicit. He called for reconsideration of *Griswold*, which protects access to contraception, and *Obergefell*, which guarantees same-sex marriage, on similar grounds (*Legal Analysis: What Dobbs Got Wrong*, 2023). The decision thus threatened not only abortion access but the entire edifice of substantive due process, the principle that certain fundamental rights are protected even if not explicitly enumerated (Cornell Law School, n.d.). For this analysis, what matters is not merely the policy outcome but the worldview it encodes. The Constitution protects those who wrote it but bodies outside that frame (including women, queer people, and people of color) are subject to the states in a way that white men are not.

The legal environment is further escalated by the proliferation of "fetal personhood" laws. Georgia's Living Infants Fairness Equality Act demonstrates how a state recognizes "unborn children" as "natural persons" at any stage of development, creating a legal conflict between the rights of the pregnant person and the state's interest in the fetus (Romo, 2019). In 2025, the consequences of fetal personhood laws became tragically clear when Adriana Smith, an Atlanta nurse and mother, was declared brain dead early in her pregnancy of her second child. Despite her condition being irreversible, the hospital kept her body on life support for months—against her family's wishes and medical judgment—to support the gestation of the fetus inside her corpse (Mador, 2025). The hospital treated Smith as what legal scholar Dorothy Roberts might call a vessel rather than a subject, where her body's value derived entirely from its capacity to gestate (D. Roberts, 1998). Here the logical endpoint of fetal personhood becomes plain: the pregnant person ceases to be a subject and becomes an object of state interest.

This logic has deep roots. *Partus sequitur ventrem* enshrined the principle that enslaved women's wombs produced property for their enslavers (Owens & Fett, 2019). The forced sterilizations of the early twentieth century that were justified by eugenic fears of unfit populations treated Indigenous, disabled, and poor women's fertility as a threat to be eliminated (Burrus, 2016). The welfare queen mythology of the late twentieth century constructed Black

mothers as parasites whose reproduction required state discipline (Garrity, 2024). In each iteration, the state has asserted its power to determine whose bodies are autonomous and whose are subject to control. The digital surveillance documented in this article is simply the latest technology for executing that project. With this legal and historical context established, we turn to scholarly literature that frames our understanding of these dynamics.

Literature Review

Scholars across disciplines have traced how the regulation of reproduction has always been central to American governance. This article draws together three bodies of work to frame the weaponization of reproductive health data as neither novel nor anomalous: Foucauldian biopolitics, critical race and feminist theory on reproductive control, and emerging scholarship on digital surveillance.

Michel Foucault's concept of biopower provides a starting point. Foucault described how modern states govern not through explicit coercion alone but through the regulation of populations at the level of life itself. Reproduction emerged as a key site for this intervention (Cisney & Morar, 2015). The population is rendered knowable, trackable, and controllable, first through censuses and clinics, now through data brokers and algorithmic inference. Recent scholarship has extended this framework to the post-Dobbs landscape. Ella Breiler's analysis of abortion laws in Texas, Mississippi, and California shows how contrasting legal frameworks produce unequal realities. Some states have built what she calls "a model built on surveillance and public enforcement," while others frame abortion as healthcare, though access remains shaped by structural inequality. Race, income level, immigration status, and access to transport all determine who can exercise reproductive autonomy and who cannot (Breiler, 2025).

Where Foucault theorized the general mechanisms of biopower, critical race and feminist scholars have specified how those mechanisms operate along lines of race and gender. Dorothy Roberts's foundational work, *Killing the Black Body*, documents a centuries-long project of regulating Black women's reproduction. This project stretches from the commodification of enslaved women's wombs to the forced sterilizations of the twentieth century to the prosecution of pregnant women today. Roberts argues that reproductive control has never been merely about abortion. It is about determining which bodies are fit to reproduce and which are not (D. Roberts, 1998). Masters' analysis of welfare reform traces how policy has consistently constructed the sexual conduct of the racialized poor mother as a primary disciplinary target, from the "welfare queen" stereotype to marriage promotion mandates embedded in federal law (Masters et al., 2014). These constructions do not simply reflect prejudice. They authorize surveillance and intervention.

The eugenic roots of this project are not distant history but living precedent. The Supreme Court's 1927 decision in *Buck v. Bell* upheld the forced sterilization of Carrie Buck and legitimated the state's power to determine who is fit to reproduce (*Buck v. Bell*, 274 U.S. 200 (1927), n.d.). Its logic echoes in contemporary fetal personhood laws, which treat pregnant people as vessels for rights that belong elsewhere. Andrew R. Lewis's analysis of Christian

fundamentalism and anti-abortion politics traces how opposition to abortion has been linked, from the early twentieth century onward, to broader anxieties about sexual morality, the family, and racial purity. The contemporary pro-life movement is heir to this fundamentalist legacy, which shapes not only its content but its political style (Lewis, 2018). The Southern Baptist Convention's recent vote opposing in vitro fertilization signals the movement's next frontier. If life begins at conception, then the destruction of embryos created during IVF is murder, and the state has an interest in regulating not just abortion but the entire reproductive process (Messerly, 2024). This logic extends seamlessly to digital surveillance. If every fertilized egg is a person, then tracking the bodies that carry them becomes not an intrusion but a protection.

The third body of literature this article engages concerns digital privacy and reproductive surveillance. The Health Insurance Portability and Accountability Act (HIPAA) has long been understood to contain significant gaps. Cohen and Mello document how HIPAA's limited scope, applying only to providers, plans, and their business associates, leaves health data generated outside traditional clinical settings entirely unprotected (Cohen & Mello, 2018). This gap has become a crisis in the post-Dobbs era. Brynn Drury's analysis of reproductive data privacy argues that Dobbs activated a pre-existing but dormant surveillance infrastructure, transforming the commercial data ecosystem into a forensic tool (Drury, 2025). Technical scholarship underscores the stakes. A seminal 2013 study by de Montjoye and colleagues found that 95% of individuals can be uniquely re-identified from location data using only four data points (de Montjoye et al., 2013). Advances in machine learning have only intensified this risk. As Gadotti and colleagues document, the concept of truly anonymized data is increasingly fragile as fragments of digital trails can be reassembled with alarming precision (Gadotti et al., 2024).

Recent empirical work confirms that these risks are not theoretical. Zadushlivy, Biviji, and Williams's 2025 analysis of reproductive health apps found that standard industry practices create a landscape where sensitive health inferences can be made at scale. These practices include pervasive data sharing with third-party advertisers and the collection of persistent identifiers like IP addresses. They conclude that user expectations for data privacy in reproductive health apps are not being met. They recommend that apps minimize data collection, exceed encryption and anonymization standards, and reduce IP address tracking (Zadushlivy et al., 2025). Yet corporate commitments to delete sensitive data have proven unreliable. A *Washington Post* investigation found that Google failed to honor its promise to automatically delete location data for abortion clinic visits in approximately half of test cases (Fowler, 2023).

What unites these literatures is an understanding that surveillance is not a technological accident but a political choice. The data broker industry collects and sells exhaustive profiles on virtually every U.S. adult, with law enforcement as a key client (Kempt, n.d.). Ayoub and Goitein detail how agencies secretly purchase data to bypass warrant requirements (Ayoub & Goitein, 2024). This is biopower operationalized through code. Bodies are rendered legible to the state not through the overseer's gaze but through the algorithm's inference.

This article builds on these scholarly foundations to make two interconnected arguments. First, the weaponization of reproductive health data after Dobbs is not a departure from

American legal tradition but its fulfillment. It is a modern manifestation of the centuries-old project to regulate bodily autonomy, particularly for women, people of color, and LGBTQ+ individuals. Second, legislative reform like the Digital Bodyguard Act is necessary but insufficient. Such statutes are tools, not solutions. They can constrain the worst excesses of the surveillance state, but they cannot resolve the deeper conflict over whether bodies are sovereign or subject. That conflict requires not just policy change but structural transformation, a project this article locates within the longer arc of struggles for reproductive justice. This scholarship illuminates the stakes of the current political battle over reproductive health data.

The Political Terrain

The battle over reproductive health data is not a dispute between competing policy preferences. It is a conflict over whether bodies are sovereign or subject, and the coalitions that face each other across this divide reflect the stakes.

On one side stand reproductive justice advocates, civil liberties organizations, and medical associations. Despite their different missions, they share a core commitment to the idea that individuals should control their own bodies and the data those bodies generate. Organizations like Pregnancy Justice have documented the crisis (“Pregnancy as a Crime: An Interim Update on the First Two Years After Dobbs,” 2025), the ACLU litigated against surveillance (*Surveillance Technologies*, n.d.), and medical organizations like The American Medical Association have grounded the argument in medical ethics (Mukkamala, 2024). In states like California (“California Code, Health and Safety Code - HSC § 123467.5,” n.d.) and New York (*SHIELD Act*, n.d.), they have translated this commitment into shield laws that block cooperation with out-of-state investigations. These are not abstract policy positions; they are assertions that some bodies—pregnant bodies, patient bodies—remain outside the state's reach.

Opposing them is a coalition unified not by a single ideology but by a shared interest in maintaining access to those bodies. For social conservatives and the architects of Project 2025, that access enables the enforcement of fetal personhood and conservative gender roles (“Project 2025 Threatens Women and Families’ Health and Freedom,” n.d.). For the data broker industry, access enables a multi-billion-dollar market in human information, where location histories and search queries are packaged and sold to the highest bidder, including law enforcement agencies that use them to bypass warrant requirements. These interests converge in practice: ideologues and federal actors create the political and legal demand for surveillance, state actors translate those ideologies into state laws and investigations that create a market for data, and the data industry supplies the commodity, sensitive data, that fuels the entire system.

This coalition's theological commitments are not mere window dressing. As Andrew Lewis documents, the Christian fundamentalist opposition to abortion has always been linked to broader anxieties about sexual morality, the family, and racial order¹. But the current movement has grown more explicit about its ambitions. Project 2025, the 920-page blueprint drafted by the

¹ (Lewis, 2018)lewislewis

Heritage Foundation for the next Republican administration, is rooted in what scholars call Christian nationalism, which is the belief that the United States is and must remain a nation governed by a particular interpretation of Christian doctrine (Kobes de Mez, 2021). Its authors want to "restore the family as the centerpiece of American life," and they define that family as a married father, mother, and children, with federal funds directed to support a "biblically based" definition of family ("Project 2025 Threatens Women and Families' Health and Freedom," n.d.).

This vision is fundamentally about the status of women. The Heritage Foundation has argued that birth control pills feminize men, pollute the groundwater with estrogen, and make women "not as attractive to men" by suppressing menstruation (Yenor, 2025). These arguments sound absurd until you recognize what they are doing. They are constructing a world in which women's primary function is biological, their bodies exist for male desire and reproduction, and any technology that disrupts that function is a threat to the social order. Project 2025 calls for the deletion of terms like "reproductive health" and "reproductive rights" from every federal rule and regulation. It proposes using the Comstock Act, a 19th-century anti-vice law, to ban the mailing of abortion medication nationwide. It frames abortion not as a medical procedure but as a "brutal act that destroys the life of an unborn child," and it treats any attempt to protect patient privacy as "ideologically motivated fearmongering." (Bernstein et al., 2024, p. 2)

The logic extends beyond abortion. Project 2025 seeks to embed fetal personhood across federal agencies, requiring that every miscarriage, stillbirth, and pregnancy loss be reported to the government. It calls for data collection on every abortion, including the mother's state of residence, the gestational age, and the reason for the procedure ("Project 2025, Explained," n.d.). This Orwellian surveillance is designed to make every pregnancy visible to the state as a tool of enforcement.

The movement's hostility to contraception, IVF, and abortion are not separate campaigns. They are a coherent project to reassert male authority over female reproduction. What we are witnessing is not merely religious conservatism but religious fundamentalism. This is a project that seeks to elevate a single, literalist interpretation of Christianity to the status of a super-right, one that overrides women's liberty and equality claims at every turn. Justice Thomas and Justice Alito are not simply conservative jurists but religious fundamentalists whose opinions in *Dobbs* reveal a willingness to impose a particular theological vision on the entire country, indifferent to the human cost (Greenhouse, 2022).

This is the worldview that animates the surveillance state this article documents. If women are divinely ordained to be wives and mothers, if their bodies exist for reproduction, then any deviation (any abortion, any contraception, any assertion of autonomy) becomes not just a policy disagreement but a theological offense. And if it is a theological offense, then surveillance is not an intrusion. It is a sacred duty.

This coalition now holds power at the federal level. The repeal of the 2024 HIPAA Reproductive Health Care Privacy Rule, the dismantling of [womenshealth.gov](https://www.womenshealth.gov), and proposed CMS health data tracking systems are not isolated actions or administrative errors. They are the

implementation of a vision in which the state's right to know supersedes the individual's right to be private, which is a vision that treats certain bodies as inherently subject to scrutiny.

Navigating this terrain requires recognizing that the conflict is not between two privacy frameworks but between two conceptions of personhood. In one, the individual is the basic unit of rights. On the other, some individuals are containers for rights that belong elsewhere—to the fetus, to the state, to the market. Bodily autonomy is the foundational right to have self-determination over one's own body and life² while fetal personhood seeks to grant fertilized eggs and zygotes full constitutional rights, effectively treating the pregnant person as a state-regulated vessel. At its core, this is a conflict over the concept of bodily ownership: bodily autonomy asserts that an individual's body is their own, while fetal personhood effectively treats the pregnant person's body as a vessel that can be regulated and controlled by the state (Valenti, 2024). This chasm is not merely philosophical but physical, underscored by the biological reality that a fetus requires gestation inside another person's body for approximately 24 weeks to achieve a chance of viability outside the womb (Breborrowicz, 2001).

At its core, then, the battle over reproductive health data is a battle over feminism itself. The conservative movement has learned that direct attacks on women's equality are politically costly, so it has developed a more sophisticated strategy. It appropriates the language of feminism while draining it of content. We hear about "women's health" from officials who gut maternal health resources (Rosenthal & Jones, 2025). We hear about "protecting women" from the same administration (*Defending Women From Gender Ideology Extremism And Restoring Biological Truth To The Federal Government – The White House*, 2025) that tracks pregnant immigrants through labor and delivery (Bhuiyan, 2025). We hear about "dignity" (*Defending Women From Gender Ideology Extremism And Restoring Biological Truth To The Federal Government – The White House*, 2025) from advocates who would force a ten-year-old rape victim to carry her attacker's child to term (Cox, 2022).

This is not hypocrisy. It is an ideology. The movement has constructed a version of conservative feminism that celebrates women's distinct caregiving capacities while insisting those capacities bind them to home and family. Women are honored so long as they accept that their highest calling is motherhood, their bodies are not their own, and their reproductive decisions belong to God, to husbands, and to the state.

The surveillance apparatus described in these pages is the enforcement mechanism for that ideology. You cannot have a world in which women are subject without the means to track them when they stray. With this political landscape in view, we can now examine the specific surveillance infrastructure that these coalitions are fighting to control or dismantle.

The Surveillance Infrastructure

This new prosecutorial reality weaponizes the digital trail people leave in their daily lives. HIPAA left a significant loophole, as it only applies to healthcare providers, health plans,

² Jessica Valenti, *Abortion: Our Bodies, Their Lies, and the Truths We Use to Win*, 1st ed. (Penguin Random House, 2024).

and their “business associates,” not to the vast ecosystem of health and wellness apps (such as Flo or Clue), search engines, social media platforms, and data brokers that purchase and aggregate user data. Consequently, the digital trail people leave (such as text messages, search histories, period-tracking app data, and location data) is now potential evidence for law enforcement (Cohen & Mello, 2018). It creates a two-tiered system for health information: data within the traditional healthcare system is protected, while identical data generated by individuals through apps, searches, and devices is left vulnerable to collection by commercial entities and, by extension, law enforcement. Geolocation data revealing visits to clinics and purchase histories for pregnancy tests is stripped of federal health privacy safeguards the moment it leaves a doctor’s office and enters the digital marketplace (Kohane, 2024).

In Nebraska, law enforcement served Meta with a warrant for the Facebook messages of Jessica and Celeste Burgess, who were charged with violating state abortion law. The company provided access to their private data, which was used in the prosecution (Jiménez, 2023). Similarly, a Texas sheriff’s office used a network of automated license plate readers to track a woman based on a log entry stating “had an abortion” (Alajaji, 2025), and in Idaho, police used cell phone location data to investigate a family traveling to Oregon for an abortion (Press, 2023). The data broker industry is central to this threat, as demonstrated when data broker Near Intelligence sold location data from nearly 600 Planned Parenthood clinics to anti-abortion groups (Gullo, 2024). Furthermore, a congressional inquiry revealed that major pharmacy chains have routinely turned over patient prescription records to law enforcement without a warrant (*Wyden, Jayapal and Jacobs Inquiry Finds Pharmacies Fail to Protect the Privacy of Americans’ Medical Records; HHS Must Update Health Privacy Rules*, 2025).

The weaponization of digital surveillance extends beyond post hoc criminal investigation to proactive real-time monitoring of vulnerable populations. A 2025 investigation revealed that U.S. Immigration and Customs Enforcement (ICE) mandates pregnant immigrants in its “Alternative to Detention” program to wear unremovable smartwatches (VeriWatch) that track their location at all times, including during labor and delivery. Hospital staff reported that this surveillance caused medical delays and profound fear, with patients terrified that removing the device for necessary procedures like C-sections would trigger their detention (Bhuiyan, 2025). This exemplifies the direct pipeline from commercial monitoring technology to state control over reproductive bodies, creating a climate where seeking essential healthcare is fraught with the threat of punitive surveillance. Here, the surveillance state’s targets are explicit: pregnant immigrants, already positioned at the intersection of multiple systems of control, are rendered trackable through their very biology. The smartwatch is not a deviation from reproductive governance but its latest technology.

The numbers tell a stark story. A September 2025 report from Pregnancy Justice documents 412 pregnancy-related criminalization cases in the first two years after Dobbs. These cases do not fall evenly across the population. More than three-quarters involve low-income people, with Black women and Indigenous women disproportionately represented. Notably, most of these prosecutions have nothing to do with fetal harm or even with completed abortions. In

nine cases, the mere consideration of an abortion—evidenced by possession of medication or attempts to obtain it—was enough to trigger charges. In 378 of those 441 charges, prosecutors were not required to prove any actual harm to a fetus or infant (“Pregnancy as a Crime: An Interim Update on the First Two Years After Dobbs,” 2025).

This data reveals something important about what these prosecutions are actually for. They do not require harm because harm is not the point. The point is surveillance itself, the assertion of state authority over bodies deemed insufficiently autonomous. This is biopower in its most literal form: the regulation of populations at the level of life (Cisney & Morar, 2015).

The clinic is not the only entry point to this surveillance system. The same logic operates with even greater force in the commercial digital ecosystem. The data broker industry, which operates largely unregulated at the federal level (*Expert Warns Data Brokers Profit from Unregulated Surveillance*, n.d.) and forms the backbone of the digital surveillance threat, aggregates and sells profiles on virtually every U.S. adult (Kempt, n.d.). The weaponization of this data is enabled by two foundational technical realities: the inherent limits of data anonymization and the power of algorithmic inference.

First, the inherent identifiability of so-called anonymized data: a seminal 2013 study found that 95% of individuals could be uniquely re-identified from their location data using only four geospatial data points (de Montjoye et al., 2013). This risk is operationalized in practices like the sale of location data from Planned Parenthood clinics (*Cellphone Location Data Used to Target Abortion Misinformation to Visitors at 600 Reproductive Health Clinics in 48 States*, 2024) and the use of automated license plate readers in abortion investigations (Alajaji, 2025). Advances in machine learning and artificial intelligence (AI) have dramatically increased this re-identification risk and enabled sophisticated data linkage and inference (Pratt et al., 2025). AI systems now amplify specific privacy threats, most notably ‘identification’—the automated linking of identity across data sources—and ‘aggregation’—the combination of disparate data points to infer sensitive information. While the 2013 study demonstrated the theoretical risk of re-identification (de Montjoye et al., 2013), modern AI-powered tools have operationalized this threat at scale (Lubarsky, 2017). Companies now use machine learning to match “anonymized” location trails from millions of devices with public records and social media check-ins (Team, 2024), effectively stripping away anonymity and allowing, for instance, the identification of every device that spent more than 30 minutes at a Planned Parenthood clinic on a given date. The power of these techniques is underscored by research indicating that with contemporary analytical methods, just four auxiliary data points can be sufficient to uniquely identify 95% of individuals within a massive dataset (Gadotti et al., 2024). In the current digital ecosystem, the concept of truly ‘anonymized’ data is increasingly fragile, as fragments of a person’s digital trail can be reassembled with alarming precision.

Second, predictive algorithms can infer sensitive health status from seemingly benign behavioral proxies, a process known as algorithmic inference. Consumer technology like the Oura Ring can detect physiological changes that identify pregnancy before most people know they are pregnant (Borges et al., n.d.). Beyond wearables, machine learning models analyze

patterns in purchase history, mobility routines, and app usage to create high-fidelity health profiles. A 2025 technical analysis of popular reproductive health apps concluded that standard industry practices—including pervasive data sharing with third-party advertisers and the collection of persistent identifiers like IP addresses—creates a landscape where sensitive health inferences can be made at scale (Zadushlivy et al., 2025). This is particularly alarming given that approximately one-third of American women use digital apps to track their menstrual cycles (Kelly & Habib, 2023). This creates a surveillance landscape where a person can be profiled and targeted without ever typing a sensitive search term. The convergence of these technical capabilities—the ability to re-identify individuals from anonymous data and to infer private health status from behavioral traces—with the post-Dobbs legal landscape transforms the commercial data ecosystem into a forensic tool. Despite the clear risks, voluntary corporate measures have proven unreliable. A Washington Post investigation, for example, found that Google failed to honor its commitment to automatically delete location data for visits to abortion clinics in approximately half of all test cases (Fowler, 2023).

The federal government has enabled this surveillance environment. The Biden administration's 2024 HIPAA Reproductive Health Care Privacy Rule was repealed by the Trump administration and subsequently vacated by the courts in *Purl v. HHS*, removing heightened federal protections (Murray & White, 2025). Furthermore, the executive branch has actively reshaped the federal information landscape; a 2025 content analysis documented the systematic removal of evidence-based maternal health resources from [womenshealth.gov](https://www.womenshealth.gov), deleting content discussing structural racism as a cause of health disparities and promoting ideologies that frame women as vulnerable, thereby creating a perceived need for the digital surveillance evidenced in cases like Burgess (Homan & Short, n.d.). This ideological framing of women as inherently vulnerable or in need of protection is not merely rhetorical; it provides a justification for the very surveillance state documented in cases like Burgess. If women are constructed as incapable of making informed reproductive decisions without state oversight, then monitoring their digital lives, whether through data broker purchases, cell phone tracking, or smartwatch mandates, becomes framed as a protective measure rather than a punitive one. Compounding the problem, new federal initiatives, like a proposed Centers for Medicare and Medicaid Services (CMS) health data tracking system, risk creating large-scale targets for law enforcement subpoenas (Lee & Bennett, 2025).

The Supreme Court's decision in Dobbs activated a pre-existing but dormant surveillance infrastructure (Drury, 2025). The convergence of state trigger bans, personhood laws, the vast, unregulated data broker industry, and an active federal campaign to obscure health information has created a legal environment where personal digital data is weaponized to investigate, prosecute, and punish individuals for their reproductive choices. The federal government must enact a uniform standard to close the HIPAA loophole and prevent health data from being used as evidence in abortion-related prosecutions. It is in response to this crisis that the Digital Bodyguard Act is proposed.

Policy Proposal: The Digital Bodyguard Act

The Digital Bodyguard Act responds to a specific failure. Current law creates a two-tiered system for health information. Data generated within a hospital is protected. The same data generated by a person on their phone is not. This distinction made little difference when reproductive healthcare was a constitutional right, but it makes all the difference now.

The Act establishes a prohibited-by-default standard for sensitive health data. This means companies cannot collect, sell, or share such data unless the user gives explicit, informed, and easily revocable consent. The burden shifts from the individuals to the corporation. Your data belongs to you unless you say otherwise, not the reverse.

The Act defines sensitive health data broadly. It covers information related to reproductive health, menstruation, fertility, pregnancy, contraception, and abortion. It also covers data inferences, which is health information derived from non-health-related data, like inferring pregnancy from sleep patterns or location history. Precise location data near healthcare facilities is explicitly included. Search history and communications related to seeking this information are protected as well.

For enforcement, the Act creates a dual mechanism. The FTC can bring actions and levy fines, while individuals can also sue for statutory damages, creating a distributed deterrent that operates even when regulators lack political will. Data covered under the Act can only be disclosed to law enforcement with a warrant based on probable cause for a crime unrelated to reproductive healthcare.

The Act's path through Congress depends on electoral outcomes that remain uncertain. In the 119th Congress, unified Republican control forecloses any possibility of advancement. Should Democrats retake the House in 2026, champions like Representative Frank Pallone (*Pallone Calls for Federal Protection of Right to Full Reproductive Health Care Including Abortion* | Congressman Frank Pallone, 2024) and Representative Sara Jacobs (*Wyden, Jayapal and Jacobs Inquiry Finds Pharmacies Fail to Protect the Privacy of Americans' Medical Records; HHS Must Update Health Privacy Rules*, 2025) could advance the bill. The Senate remains a steeper challenge; even with a House majority, the filibuster and Republican control of key committees would require moderate Republican support that is unlikely to materialize.

This political reality does not diminish the Act's importance. It clarifies it. Legislation alone cannot resolve a conflict rooted in competing conceptions of personhood. The Act is a tool, not a solution—a necessary intervention that, even if enacted, would require constant defense.

Implementation and Enforcement

A law is only as strong as its enforcement. The Digital Bodyguard Act's mechanisms matter not only for what they accomplish but for what they assert. A private right of action says that a person can sue when their data is weaponized. An exclusionary rule says evidence obtained through illegal surveillance cannot be used against you. These provisions affirm that

your body's data is yours in a way that current law denies. That affirmation matters precisely because the legal system has so often treated some bodies as if they belonged to someone else.

The FTC will serve as the primary enforcer. Within 180 days of enactment, it must issue rules clarifying key definitions like “data inferences” and “granular consent”. Its Bureau of Consumer Protection will establish a dedicated task force to proactively monitor the data broker market, using subpoena power to compel testimony and document production. Fines will be levied on a per-violation, per-day basis, set high enough to deter profitable noncompliance. The Act requires dedicated funding, modeled on the \$1 billion over ten years proposed in Senator Warren's Health and Location Data Protection Act, to prevent enforcement decay.

For data brokers and tech companies, the Act creates a voluntary certification program. Companies that demonstrate compliance with the Act's standards benefit from a presumption of compliance and a market differentiator. Major data brokers must undergo independent, third-party audits every two years. These audits examine not only data inventories but also the models used for advertising targeting and user categorization, ensuring they do not illegally infer sensitive health data.

For law enforcement, the Act imposes clear limits. No local, state, or federal agency may use data under this Act to investigate or prosecute reproductive healthcare. Any evidence obtained in violation of this rule is subject to the exclusionary rule. This creates a powerful legal disincentive for prosecutors and police who might otherwise seek out this data.

Success will be measured by multiple metrics. A year-over-year decrease in data-based prosecutions for abortion-related investigations will signal real-world impact. A reduction in the volume of sensitive health datasets available for purchase on the data broker market will indicate that the Act is working before cases ever reach court. The FTC will track enforcement actions, private lawsuits, and audit results. It will maintain a public dashboard displaying key metrics, ensuring accountability to the public and providing data for researchers and advocates.

Every three years, the FTC and GAO will report to Congress on the Act's impact. This review will assess performance metrics, evaluate enforcement strategies, and identify emerging loopholes. Based on the findings, Congress can expand protected categories, strengthen penalties, or close new gaps. The goal is a law that evolves alongside the surveillance technologies it seeks to constrain.

The primary challenge will be sustaining political will and funding across administrations. Sunset provisions or locked-in trust funds could insulate the FTC from political pressure. Continuous vigilance will be required to adapt to rapidly evolving new privacy threats. But these challenges, however real, do not negate the necessity of the attempt.

Conclusion

The Digital Bodyguard Act is a response to a specific crisis, but the crisis it addresses is not an anomaly but the latest iteration of a project as old as the nation itself. From the commodification of enslaved women's wombs to the forced sterilization of the "unfit" to the prosecution of pregnant women today, the American state has consistently asserted its power to

determine whose bodies are autonomous and whose are subject to control. The tools have changed but the project endures.

What *Dobbs* activated was not a new surveillance infrastructure but a pre-existing one. The data broker industry, the period-tracking apps, the license plate readers, the smartwatches on pregnant immigrants' wrists—these were not built in response to the decision. They were built for other purposes: profit, convenience, efficiency, but they were always available for this one, always waiting to be repurposed.

The women in the Pregnancy Justice report, the immigrants tracked through labor and delivery, Adriana Smith kept on life support against her family's wishes—these are not cautionary tales about what might happen if we fail to act. They are evidence of what is already happening. They are the present, not the future. And they are not distributed evenly across the population. They are concentrated among those who have always been targets: Black women, Indigenous women, women whose bodies were never fully theirs to begin with.

That is why the Digital Bodyguard Act, however necessary, is not enough. It can constrain the worst excesses of the surveillance state. It can deter some prosecutors, slow some investigations, and protect some data. But it cannot resolve the deeper conflict over whether bodies are sovereign or subject. It cannot overturn *Buck v. Bell* or the logic that produced it. It cannot dismantle the structures of patriarchy and white supremacy that have shaped American law from the beginning.

What it can do is draw a line. It can say that some data is off limits, that some bodies are not evidence, that the state's right to know stops at the bedroom door, the clinic door, the phone screen. It can create a space, however small and contested, where people can seek reproductive healthcare without becoming visible to the state. It can make the surveillance apparatus work a little harder, cost a little more, catch a little less. This matters for the woman considering an abortion in Texas, for the immigrant tracked through labor in an ICE facility, for the girl who does not yet know she will need care. That space can mean the difference between prosecution and privacy, between being a subject and being a vessel. The Act cannot guarantee those outcomes, but it can make them possible.

The longer project remains. It is the project Dorothy Roberts named, the project feminists have fought for across generations. From suffragists who knew the vote was never enough, to welfare rights activists who insisted poor mothers deserved dignity, to reproductive justice organizers who taught us that abortion access without childcare, housing, and healthcare is freedom for some but not for all. It is the project of insisting that all bodies are sovereign, that no body is subject, that the state's power stops where the skin begins.

That project will not be completed by any single statute. Not by the Equal Rights Amendment, though we should have ratified it decades ago. Not by the Digital Bodyguard Act, though we need it desperately. These are tools, not solutions. They create footholds. They draw lines. But the work is bigger than any law. It requires organizing, litigating, legislating, and agitating for as long as the project it opposes endures. And that opposing project has centuries of momentum. It has the Constitution, interpreted by men who wrote it for themselves. It has data

brokers, surveillance technology, and centuries of legal precedent treating some bodies as subject rather than sovereign. So the question is not whether the fight will be hard. The question is whether we are willing to fight it.

So long as the fundamental principles of bodily autonomy and true gender equality remain unenshrined in our constitution, the state will always find new tools. Empowered by patriarchal and religious imperatives, it will reach for dormant archaic laws and build digital dragnets to police, to punish, and to perpetuate its power. The pursuit of this statute is necessary precisely because the more profound, structural reforms that would guarantee equality remain, for now, a promise denied.

The promise can be redeemed, the line can be moved. What was unthinkable becomes thinkable becomes law becomes precedent becomes tradition. The project of control is never complete. Neither is the project of freedom. The algorithm is new, the womb is not. The state's reach is long. The body endures.

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